

GOVERNMENT HEALTH INSURANCE SCHEMES IN INDIA

A Comprehensive Research Paper

Focus on Maharashtra, Telangana/Andhra Pradesh, Kerala, and Jammu & Kashmir

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ABSTRACT

This research paper presents a comprehensive analysis of major government-sponsored health insurance schemes in India, with specific focus on four landmark state and union territory initiatives: (1) Mahatma Jyotiba Phule Jan Arogya Yojana (MJPJAY) in Maharashtra, (2) Aarogyasri Health Scheme in Telangana and Andhra Pradesh, (3) Karunya Arogya Suraksha Padhathi (KASP) and MEDISEP in Kerala, and (4) PM-JAY SEHAT in Jammu & Kashmir.

The study examines the structural frameworks, coverage parameters, eligibility criteria, financial allocations, and implementation mechanisms of these schemes. Through comparative analysis, the paper identifies best practices, systemic challenges, and policy recommendations for strengthening India's public health insurance architecture. The research draws upon official government data, academic literature, and implementation reports to provide an evidence-based assessment of how these schemes contribute to Universal Health Coverage (UHC) goals and reduce catastrophic out-of-pocket health expenditure (OOPE) among vulnerable populations.

Key findings indicate that while these schemes have significantly expanded healthcare access for economically disadvantaged populations, challenges persist in areas of beneficiary identification, claims processing, hospital empanelment quality, and integration with primary healthcare. The paper concludes with actionable recommendations for policy harmonization, digital infrastructure enhancement, and sustainable financing mechanisms.

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1. INTRODUCTION

India's healthcare landscape has undergone a significant transformation over the past two decades, marked by the proliferation of government-sponsored health insurance schemes (GSHIS) aimed at achieving Universal Health Coverage (UHC). Despite being one of the world's fastest-growing economies, India spends only about 2% of its GDP on healthcare, significantly lower than the WHO-recommended 5%. This underfunding has historically resulted in high out-of-pocket expenditure (OOPE), with households bearing approximately 62% of total health expenditure.

The Government of India and various state governments have launched numerous health insurance initiatives to mitigate catastrophic health expenditure and ensure equitable access to quality healthcare. These schemes represent a paradigm shift from out-of-pocket payments to pooled risk financing, particularly for secondary and tertiary care services that impose the greatest financial burden on poor and vulnerable households.

This research paper focuses on four exemplary schemes that represent diverse implementation models across India's federal structure:

1. Mahatma Jyotiba Phule Jan Arogya Yojana (MJPJAY) - Maharashtra's flagship scheme, originally launched as Rajiv Gandhi Jeevandayee Arogya Yojana in 2012, representing one of India's earliest state-level health insurance programs.
2. Aarogyasri Health Scheme - Operating in both Telangana (as Rajiv Aarogyasri) and Andhra Pradesh (as Dr. YSR Aarogyasri), this scheme pioneered the trust-based model of public-funded health insurance in India when originally launched in 2007.
3. Karunya Arogya Suraksha Padhathi (KASP) and MEDISEP - Kerala's dual-track approach covering both BPL/vulnerable populations (through KASP) and government employees/pensioners (through MEDISEP), representing comprehensive coverage across different socioeconomic segments.
4. PM-JAY SEHAT - Jammu & Kashmir's universal health coverage extension of the national Ayushman Bharat PM-JAY, notable for covering all residents regardless of income or socioeconomic status.

These schemes collectively cover millions of beneficiaries and represent distinct approaches to healthcare financing, beneficiary identification, provider payment mechanisms, and integration with existing health systems. Understanding their design, implementation, and outcomes provides critical insights for India's ongoing UHC journey.

2. HISTORICAL EVOLUTION OF HEALTH INSURANCE IN INDIA

India's journey toward publicly funded health insurance can be traced through several landmark initiatives:

The Rashtriya Swasthya Bima Yojana (RSBY), launched in 2008, marked India's first national-level attempt to provide health insurance to Below Poverty Line (BPL) families. Covering approximately 55 million households with a sum insured of Rs. 30,000 per family, RSBY introduced smart card-based cashless hospitalization. However, the scheme suffered from low enrollment among genuinely poor households, particularly Scheduled Tribes and Muslims, and was criticized for its low coverage ceiling.

The launch of Aarogyasri in undivided Andhra Pradesh in 2007 represented a watershed moment, pioneering the trust-based model with significantly higher coverage of Rs. 2 lakh per family. This model demonstrated that state governments could effectively implement large-scale health insurance without commercial insurance intermediaries.

The Ayushman Bharat Pradhan Mantri Jan Arogya Yojana (AB-PMJAY), launched on September 23, 2018, represents the most ambitious expansion of publicly funded health insurance globally. Targeting over 10.74 crore poor and vulnerable families (approximately 50 crore beneficiaries) based on SECC 2011 data, PM-JAY provides Rs. 5 lakh coverage per family per year for secondary and tertiary care. The scheme subsumed RSBY and has been adopted by most states, either in pure PM-JAY form or as integrated state-specific variants.

In September 2024, the Union Cabinet approved extending PM-JAY coverage to all individuals aged 70 years and above, irrespective of income, through the Vay Vandana Card, further expanding the scheme's universal reach. As of 2026, over 8.19 crore people have benefited from treatment under the scheme, with government expenditure exceeding Rs. 1,26,000 crore.

The evolution reflects a clear trajectory: from small-scale, low-coverage pilots to comprehensive, high-coverage schemes; from BPL-only targeting to broader vulnerability-based inclusion; and from insurance-company-mediated models to trust-based direct implementation.

3. SCHEME-WISE DETAILED ANALYSIS

3.1 Mahatma Jyotiba Phule Jan Arogya Yojana (MJPJAY) - Maharashtra

3.1.1 Background and Launch

The Mahatma Jyotiba Phule Jan Arogya Yojana (MJPJAY) was originally launched as the Rajiv Gandhi Jeevandayee Arogya Yojana on July 2, 2012, in eight districts of Maharashtra. Following its success, the scheme was expanded to all 35 districts of the state by November 21, 2013. On April 1, 2020, the scheme was integrated with Ayushman Bharat-Pradhan Mantri Jan Arogya Yojana (AB-PMJAY) to form the Integrated MJPJAY and AB-PMJAY program.

The scheme is named after Mahatma Jyotirao Phule, the renowned social reformer who worked tirelessly for the upliftment of marginalized communities in Maharashtra. It is implemented by the MJPJAY Society under the Directorate of Health Services, Government of Maharashtra.

3.1.2 Coverage and Benefits

MJPJAY provides comprehensive health coverage with the following key features:

Coverage Amount: The scheme offers cashless treatment coverage up to Rs. 1.5 lakh per family per year on a floater basis. This means the coverage can be utilized entirely by one family member or shared among all members. For renal transplant cases, the coverage limit is enhanced to Rs. 2.5 lakh per family per year. In December 2015, the government further increased the kidney transplant expense limit from Rs. 2.50 lakh to Rs. 3 lakh, including medical check-up of both donor and receiver.

Procedures Covered: The scheme covers 996 medical and surgical procedures with 121 follow-up procedures. Out of these, 131 procedures are exclusively performed in government hospitals. The coverage spans multiple specialties including orthopaedic surgery, general surgery, ENT surgery, gynaecology and obstetrics, cardiac and cardiothoracic surgery, ophthalmology, pediatric surgery, radiation surgery, and plastic surgery.

Cashless Treatment: Beneficiaries can avail cashless treatment at over 4,000 empaneled hospitals across Maharashtra, including both government and private facilities. The state government pays an insurance premium of Rs. 333 plus taxes per year per beneficiary family to the National Insurance Company.

Pre-existing Diseases: The scheme covers pre-existing diseases and ailments from day one, with no waiting period. This is particularly significant for chronic conditions like diabetes (advanced stages such as diabetic retinopathy and diabetic ketoacidosis), heart diseases, and kidney ailments.

Follow-up Care: Beneficiaries can avail free follow-up treatments, consultations, and diagnoses for up to 10 days after discharge from the hospital.

Free Health Camps: Network hospitals conduct free health camps to facilitate early detection and preventive care

among beneficiary populations.

3.1.3 Eligibility Criteria

Eligibility for MJPJAY is determined based on socioeconomic criteria:

Income Criteria: Individual applicants must have an annual income of less than Rs. 1 lakh. Families falling under specific distressed districts of Maharashtra are prioritized.

District Coverage: The scheme initially targeted 8 districts but now covers all 35 districts of Maharashtra. Special consideration is given to families in agriculturally distressed districts.

Family Coverage: The scheme operates on a family basis, covering all eligible family members under a single policy. Family members of the primary applicant can enjoy coverage benefits.

Identification: Beneficiary families are issued an MJPJAY card or RSBY card to avail benefits at empaneled hospitals.

Exclusions: While comprehensive, the scheme does not cover treatments available at primary health centers or ordinary hospitals for common ailments. For instance, pneumonia is not covered, but advanced forms like lobar pneumonia and bronchopneumonia are covered. Similarly, basic hernia and appendicitis are excluded, but diaphragmatic hernia and appendicular perforation are included.

3.1.4 Implementation Mechanism

The MJPJAY Society, headquartered at the ESIC Hospital in Worli, Mumbai, oversees scheme implementation. The toll-free helpline (1800-233-2200) provides beneficiary support.

The scheme operates through a network of empaneled public and private hospitals. Beneficiaries present their MJPJAY card at network hospitals for eligibility verification. After verification, hospitals provide cashless treatment and submit claims to the MJPJAY Society for reimbursement.

The integration with AB-PMJAY in 2020 created a unified platform, allowing beneficiaries to access both state and central scheme benefits seamlessly. This integration expanded the hospital network and standardized treatment packages across the combined program.

3.1.5 Impact and Outcomes

MJPJAY has been instrumental in providing financial protection to BPL and APL families in Maharashtra who previously struggled to afford medical treatments. The scheme's focus on high-cost surgeries and tertiary care has significantly reduced catastrophic health expenditure among enrolled households.

Key achievements include:

- Coverage of over 971 medical procedures, making it one of the most comprehensive state-level schemes

- Network of 4,000+ empaneled hospitals ensuring geographic accessibility
- Special enhanced coverage for renal transplants, addressing a critical healthcare need
- No waiting period for pre-existing conditions, ensuring immediate access for chronic disease patients
- Free health camps promoting preventive healthcare in rural and underserved areas

However, challenges remain in reaching the most marginalized communities, particularly in tribal areas and among migrant populations who may lack proper documentation for enrollment.

3.2 Aarogyasri Health Scheme - Telangana and Andhra Pradesh

3.2.1 Background and Evolution

The Aarogyasri Health Care Scheme represents one of India's most influential state-level health insurance initiatives. Originally launched in undivided Andhra Pradesh in 2007 by then Chief Minister Dr. Y. S. Rajasekhara Reddy, the scheme pioneered the trust-based model of publicly funded health insurance. Following the bifurcation of Andhra Pradesh in 2014, the scheme continues under different names and administrative structures in both successor states.

In Telangana, the scheme operates as Rajiv Aarogyasri (also referred to as Aarogyasri Telangana), administered by the Rajiv Aarogyasri Health Care Trust under the Department of Health and Family Welfare. In Andhra Pradesh, it is implemented as Dr. YSR Aarogyasri, building upon and expanding the original framework. The scheme was previously known as NTR Aarogyasri in Andhra Pradesh, named after the late Chief Minister N. T. Rama Rao who initiated the original concept.

The Aarogyasri model has served as a template for numerous other state health insurance schemes across India, demonstrating the viability of large-scale, trust-based healthcare financing.

3.2.2 Coverage and Benefits

The Aarogyasri scheme offers one of the most comprehensive coverage packages among Indian state health insurance programs:

Coverage Amount: In Telangana, eligible beneficiaries can access free medical treatment of up to Rs. 10 lakh per family per year under Rajiv Aarogyasri. This is among the highest coverage ceilings in India. In Andhra Pradesh, Dr. YSR Aarogyasri provides similarly extensive coverage with expanded treatment packages.

Procedures Covered: The Telangana scheme covers 1,672 medical procedures across 21 specialties. In Andhra Pradesh, the coverage extends to over 1,000 procedures, including critical care, oncology, neurosurgery, and organ-related surgeries. The scheme specializes in secondary and tertiary care, covering major surgeries and critical illnesses rather than primary care or minor ailments.

Specialized Treatments: Coverage includes:

- Heart problems: Angioplasty, bypass surgery, valve replacement
- Kidney problems: Stones, dialysis, and transplantation
- Cancer: Radiation therapy, chemotherapy, and surgical procedures
- Orthopaedic conditions: Joint replacement and spinal surgery
- Neurological conditions: Parkinson's disease, stroke, multiple sclerosis
- Respiratory conditions: COPD and severe asthma
- Gastrointestinal diseases: Crohn's disease, ulcerative colitis
- Cochlear implant surgery with Auditory Verbal Therapy for children under three years (up to Rs. 6.50 lakh per case)

Cashless Treatment: The scheme provides fully cashless treatment at empaneled government and private hospitals. Beneficiaries use their AarogyaSri card for eligibility verification, after which hospitals coordinate directly with the trust for authorization and settlement.

Family Coverage: The scheme covers all eligible family members under a single household coverage, linked to the ration card or government database. There is no restriction on family size.

Follow-up Care: The scheme includes post-treatment follow-up services, diagnostics, and medicines for approved treatments as per scheme guidelines.

3.2.3 Eligibility Criteria

Eligibility for AarogyaSri is primarily based on socioeconomic status:

BPL and Low-Income Families: The scheme targets Below Poverty Line (BPL) families and other eligible low-income groups identified through state government databases. In Telangana, beneficiaries are identified through the Food Security Card and Civil Supplies Department database.

Residency: Applicants must be residents of Telangana or Andhra Pradesh, as applicable. The scheme is state-specific and not automatically portable across state boundaries.

Documentation: Required documents typically include:

- Aadhaar card for identity verification
- Proof of residence
- Income proof or BPL certificate
- Ration card or Food Security Card
- Bank statements where applicable

Aarogyamithra Support: Dedicated help desks called Aarogyamithras are available at Primary Health Centers (PHCs) and empaneled hospitals to assist beneficiaries throughout the process, from eligibility verification to treatment coordination.

Exclusions: The scheme does not cover every illness or treatment. Procedures outside approved treatment packages, non-covered conditions, and expenses not meeting scheme guidelines are excluded. Outpatient procedures and minor ailments are generally not covered to focus resources on life-threatening diseases.

3.2.4 Implementation Mechanism

The Aarogyasri scheme operates through a sophisticated public-private partnership model:

Trust-Based Administration: Unlike insurance-company-mediated models, Aarogyasri operates through state-level trusts (Rajiv Aarogyasri Health Care Trust in Telangana and the respective state health authority in Andhra Pradesh). This eliminates insurance company margins and administrative overhead, directing more funds toward actual healthcare delivery.

Digital Systems: Both states have implemented end-to-end digital processes for beneficiary verification, treatment authorization, and claim settlement. This minimizes delays and improves monitoring of services.

Hospital Network: The scheme maintains a vast network of government hospitals and empanelled private hospitals. In Andhra Pradesh, the network is particularly extensive, ensuring accessibility even in remote regions.

Pre-authorization: For major surgeries and critical care treatments, pre-authorization from the scheme trust is mandatory. This ensures that treatments are medically necessary and within approved package limits.

Package-Based Payments: Treatment is subject to standardized packages containing expenses for surgery, hospital stay, medication, diagnostics, and follow-up treatment. This transparency prevents overcharging by hospitals.

Claim Settlement: After treatment completion, empanelled hospitals submit claim packages including treatment summaries, discharge reports, medical bills, and diagnostic reports. Once verified, the trust directly reimburses hospitals for approved expenses, typically within a few days to a couple of weeks.

Help Desk Support: Dedicated Aarogyasri help desks at empaneled hospitals guide beneficiaries through eligibility verification, documentation, and treatment coordination.

3.2.5 Impact and Outcomes

The Aarogyasri scheme has had transformative impact on healthcare access in both states:

Financial Protection: By providing coverage up to Rs. 10 lakh per family, the scheme has virtually eliminated catastrophic health expenditure for enrolled BPL families requiring major medical interventions.

Healthcare Utilization: The scheme has significantly increased utilization of tertiary care services among economically disadvantaged populations who previously avoided or delayed treatment due to cost concerns.

Private Sector Engagement: By empaneling private hospitals, the scheme has leveraged private sector capacity while regulating costs through package-based payments. This has been particularly effective in addressing capacity constraints in government hospitals.

Model Replication: The AarogyaSri trust-based model has influenced the design of health insurance schemes across India, including the national PM-JAY program.

Challenges: Despite its success, the scheme faces challenges including:

- Interstate portability limitations, restricting beneficiary access when traveling outside the state
- Package rate disputes with private hospitals leading to periodic empanelment issues
- Potential for fraud claims requiring robust verification mechanisms
- Exclusion of outpatient care, leaving significant OOPe for chronic disease management
- Variations in coverage and implementation between Telangana and Andhra Pradesh post-bifurcation

3.3 Karunya Arogya Suraksha Padhathi (KASP) & MEDISEP - Kerala

3.3.1 Background and Context

Kerala represents a unique case in India's health insurance landscape, operating a dual-track approach that covers both vulnerable populations and government employees through distinct but complementary schemes.

Karunya Arogya Suraksha Padhathi (KASP) was launched in 2017 as Kerala's converged health insurance scheme, replacing multiple earlier programs including the Rashtriya Swasthya Bima Yojana (RSBY), Comprehensive Health Insurance Scheme (CHIS), Senior Citizen Health Insurance Scheme (SCHIS), and Karunya Benevolent Fund (KBF). Kerala signed an agreement with the National Health Authority (NHA) on October 31, 2018, to implement PM-JAY in the state as KASP. From July 1, 2020, the scheme is implemented directly by the Government of Kerala under trust mode through the State Health Agency (SHA).

MEDISEP (Medical Insurance Scheme for State Employees and Pensioners) was launched in July 2022 to provide comprehensive health insurance coverage to all serving state government employees, pensioners, and their dependents. Phase 2 of MEDISEP was launched in February 2026 with enhanced benefits.

Additionally, in March 2026, the Kerala government approved a new Universal Health Coverage scheme called "Health Protection for All" to cover people not included in existing programs, reflecting the state's commitment to achieving complete health coverage.

3.3.2 KASP - Coverage and Benefits

KASP provides comprehensive coverage with several distinctive features:

Coverage Amount: KASP provides a health cover of Rs. 5 lakh per family per year for secondary and tertiary care hospitalization. This aligns with the national PM-JAY coverage ceiling.

Beneficiary Base: The scheme targets over 42 lakh poor and vulnerable families (approximately 64 lakh beneficiaries) forming the bottom 40% of Kerala's population.

Procedures Covered: Services include approximately 1,573 procedures covering all treatment-related costs including drugs, supplies, diagnostic services, physician's fees, room charges, surgeon charges, OT and ICU charges, and medical implantation services where necessary.

Pre and Post-Hospitalization: The scheme covers up to 3 days of pre-hospitalization and 15 days of post-hospitalization expenses such as diagnostics and medicines.

Portability: Benefits are portable across the country, allowing beneficiaries to visit any empaneled public or private hospital in India for cashless treatment.

No Restrictions: There is no restriction on family size, age, or gender. All pre-existing conditions are covered from day one.

OPD Coverage: Unlike most Indian health insurance schemes that exclude outpatient care, KASP includes OPD follow-ups for chronic diseases like cancer and diabetes. This unique feature significantly reduces overall out-of-pocket expenditure.

Hospital Network: More than 1,000 hospitals are empaneled across Kerala and India, including both public and private facilities. Government hospitals are reimbursed at par with private hospitals.

Implementation: The scheme is implemented through the State Health Agency (SHA) with claims serviced by Heritage Health Insurance Pvt Ltd as the current TPA/ISA.

3.3.3 MEDISEP - Coverage and Benefits

MEDISEP represents Kerala's approach to covering the organized sector workforce:

Coverage Amount: Phase 2 of MEDISEP (launched February 2026) offers treatment coverage of up to Rs. 5 lakh per year, increased from the earlier cap of Rs. 3 lakh in Phase 1.

Beneficiaries: The scheme covers all serving state government employees, service and family pensioners, employees and pensioners of universities and local self-government institutions, and their dependents. This includes newly recruited employees, part-time employees, and all staff of aided schools and colleges.

Premium: Members pay a monthly premium of Rs. 687 (annual premium of Rs. 8,244) deducted directly from salary or pension.

Treatment Packages: Phase 2 includes 2,516 treatment packages (up from 1,920 in Phase 1), aligned with the National Health Benefit Package 2022. This includes medical and surgical procedures, with even knee and hip replacement surgeries now available in private hospitals.

Cashless Treatment: Cashless treatment is available at all empanelled hospitals across the state and beyond.

Implementing Agency: The scheme is implemented by Oriental Insurance Company under contract with the Kerala Government.

Pre-existing Conditions: No medical examination is required for joining, and pre-existing conditions are generally covered from the start.

Comprehensive Coverage: The scheme covers a wide range of medical and surgical procedures, including critical and specialized treatments, reducing financial strain caused by high medical costs.

3.3.4 Eligibility and Implementation

KASP Eligibility:

- Families identified as poor and vulnerable based on SECC 2011 data and state-specific criteria
- Families previously covered under RSBY, CHIS, SCHIS, and KBF
- Bottom 40% of Kerala's population
- No income criteria for the new Universal Health Coverage scheme (2026)

MEDISEP Eligibility:

- Serving state government employees (permanent, contractual, part-time)
- Retired government employees and pensioners
- Dependents: spouse, children, and recognized dependents
- Teaching and non-teaching personnel of aided schools and colleges

- Family pensioners

Implementation Structure:

Kerala's health insurance architecture is managed through the State Health Agency (SHA) with convergence of multiple schemes. The state has moved toward trust-mode implementation, reducing dependence on commercial insurance intermediaries. The recent approval of "Health Protection for All" in March 2026 aims to fill remaining coverage gaps, ensuring no family is left without treatment assistance.

3.3.5 Impact and Kerala Model

Kerala's approach to health insurance has yielded notable outcomes:

Lowest OOPE in India: Kerala has the lowest out-of-pocket health expenditure in India at approximately 35% of total health expenditure (compared to 62% nationally), largely attributable to comprehensive insurance coverage including OPD services.

Comprehensive Coverage: With KASP covering 42 lakh families and MEDISEP covering government employees, combined with the new Universal Health Coverage scheme, Kerala is moving toward true universal coverage.

Chronic Disease Management: The inclusion of OPD follow-ups for chronic diseases under KASP is a model feature that other states could emulate. This addresses a major gap in most Indian health insurance schemes.

Fiscal Sustainability: The inclusion of OPD care, while beneficial for beneficiaries, creates significant fiscal burden on the state. Kerala spends significantly more on healthcare per capita compared to many other states.

Equity: The dual-track approach ensures both vulnerable populations and government employees have access to quality healthcare, though some gaps remain for informal sector workers not covered by either scheme.

Tribal Outreach: Mobile clinics in tribal areas help extend scheme benefits to hard-to-reach populations, addressing geographic inequities in healthcare access.

3.4 PM-JAY SEHAT - Jammu & Kashmir

3.4.1 Background and Launch

The Ayushman Bharat Pradhan Mantri Jan Arogya Yojana SEHAT (AB-PMJAY SEHAT) scheme was launched on December 26, 2020, as a special extension of the national PM-JAY program for the Union Territory of Jammu and Kashmir. The term SEHAT stands for Social Endeavour for Health and Telemedicine.

This initiative is significant as it represents one of India's few truly universal health coverage schemes, extending benefits to every family in Jammu and Kashmir regardless of income level or socioeconomic status. Unlike the national PM-JAY which targets only the bottom 40% of households based on SECC 2011 data, SEHAT covers all eligible residents of the UT.

The scheme is implemented by the J&K State Health Agency (SHA) with Bajaj Allianz General Insurance Company as the insurance partner. It operates in insurance mode, distinguishing it from the trust-based models seen in some other states.

3.4.2 Coverage and Benefits

PM-JAY SEHAT offers comprehensive universal coverage with the following features:

Coverage Amount: The scheme provides cashless health insurance of up to Rs. 5 lakh per family per year on a floater basis. There is no cap on the number of family members or their age.

Universal Eligibility: All eligible residents of Jammu and Kashmir are covered under the scheme, irrespective of income level or socioeconomic category, subject to beneficiary verification and successful enrollment.

Comprehensive Protection: The scheme covers listed secondary and tertiary hospitalization treatments, subject to package rules and minimal exclusions.

Pre and Post-Hospitalization: Includes eligible expenses up to 3 days before hospitalization and 15 days after hospitalization.

Pre-existing Diseases: All eligible pre-existing conditions are covered from day one, with no waiting period.

No Age or Family Size Limit: Unlike many schemes that cap family size, SEHAT imposes no restrictions, making it particularly beneficial for large families.

National Portability: Beneficiaries can receive treatment at empaneled hospitals across India, not just within Jammu and Kashmir. This national portability is especially valuable given the UT's limited tertiary care infrastructure.

Public and Private Hospitals: Public hospitals are deemed empanelled by default. Private healthcare facilities are

empanelled based on defined criteria including specialty-specific requirements.

SEHAT Golden Card: Beneficiaries receive a SEHAT Golden Card after completing identity verification and e-KYC at Common Service Centers (CSCs) or authorized enrollment centers.

3.4.3 Eligibility and Enrollment

Eligibility Criteria:

- All residents of Jammu and Kashmir are eligible, irrespective of income
- Subject to beneficiary identification, eligibility verification, and successful enrollment
- Aadhaar serves as the primary mode of personal identification
- Alternative documents accepted include voter ID and ration card

Enrollment Process:

1. Visit a CSC or authorized enrollment center
2. Complete identity verification using Aadhaar, voter ID, or ration card
3. Complete e-KYC process
4. Upon eligibility confirmation, the SEHAT Golden Card is generated
5. Card enables cashless treatment at empaneled hospitals

Helpline: 14555 provides beneficiary support and information.

Important Distinction: PM-JAY SEHAT should not be confused with the Mukh Mantri SEHAT Yojana or Sarbat Sehat Bima Yojana, which are different schemes with separate eligibility criteria and benefits.

3.4.4 Implementation and Impact

Implementation Features:

Insurance Mode: Unlike trust-based schemes, SEHAT operates through an insurance company (Bajaj Allianz), which handles claims processing and hospital reimbursements. This brings professional insurance administration but also adds intermediary costs.

Digital Infrastructure: The scheme leverages the national PM-JAY digital infrastructure for beneficiary verification, e-cards, and hospital empanelment.

Telemedicine Integration: True to its name (Social Endeavour for Health and Telemedicine), the scheme integrates telemedicine services to improve access in remote areas of J&K.

Challenges and Context:

Geographic Challenges: Jammu and Kashmir's difficult terrain and harsh winters create unique healthcare access challenges. The national portability feature helps by allowing beneficiaries to seek treatment outside the UT when local facilities are inadequate.

Security Situation: The region's security situation occasionally disrupts healthcare delivery, making robust insurance coverage even more critical.

Infrastructure Gaps: Limited tertiary care infrastructure within the UT means beneficiaries often need to travel to other states for specialized treatment, making national portability a crucial feature.

Awareness and Enrollment: Despite universal eligibility, ensuring 100% enrollment and awareness remains a challenge, particularly in remote and conflict-affected areas.

Outcomes:

SEHAT has provided a crucial safety net for all residents of J&K, eliminating the financial barrier to secondary and tertiary healthcare. The scheme is particularly significant given the UT's economic challenges and the vulnerability of its population to health emergencies.

4. COMPARATIVE ANALYSIS

This section presents a systematic comparison of the four schemes across key parameters to identify best practices, gaps, and opportunities for cross-learning.

4.1 Comparative Table: Key Parameters

Parameter	MJPJAY (MH)	AarogyaSri (TS/AP)	KASP/MEDISEP (KL)	PM-JAY SEHAT (J&K)
Launch Year	2012 (as RGJAY)	2007 (undivided AP)	2017 (KASP), 2022 (MEDISEP)	September 2020
Coverage/Family	Rs. 1.5 lakh (Rs. 2.5L renal)	Up to Rs. 10 lakh	Rs. 5 lakh (both schemes)	Rs. 5 lakh
Target Group	BPL & low-income	BPL & low-income	BPL/vulnerable + Govt employees	All residents (Universal)
Procedures Covered	~996 procedures	1,672 (TS), 1,000+ (AP)	~1,573 (KASP), 2,516 (MEDISEP)	1,500+ (PM-JAY packages)
Implementation Model	Insurance (National Insurance)	Trust-based	Trust-based (KASP), Insurance (MEDISEP)	Insurance (MEDISEP) Allianz
Beneficiary Premium	Nil (Govt pays Rs. 333/family)	Nil (Fully govt-funded)	Nil (KASP), Rs. 687/month (MEDISEP)	Nil (MEDISEP)
Family Size Limit	No limit	No limit	No limit	No limit
Pre-existing Diseases	Covered from Day 1	Covered from Day 1	Covered from Day 1	Covered from Day 1
OPD Coverage	Not covered	Not covered	Chronic disease OPD (KASP)	Not covered
National Portability	Limited (within MH)	Limited (within state)	Yes (KASP), Limited (MEDISEP)	Yes (All India)
Empaneled Hospitals	4,000+ (MH)	Extensive network	1,000+ (Kerala & India)	PM-JAY network (All India)

4.2 Strengths and Weaknesses Analysis

4.2.1 MJPJAY (Maharashtra)

Strengths:

- One of India's longest-running state health insurance schemes with proven track record
- Extensive hospital network (4,000+) ensuring geographic accessibility
- Special enhanced coverage for renal transplants (Rs. 2.5-3 lakh)
- Integration with AB-PMJAY creating unified beneficiary experience
- No waiting period for pre-existing conditions
- Free health camps promoting preventive care

Weaknesses:

- Lower coverage ceiling (Rs. 1.5 lakh) compared to other schemes
- Limited interstate portability
- Exclusion of primary care and common ailments
- Income ceiling (Rs. 1 lakh) may exclude marginally poor families
- Dependence on commercial insurance company adds administrative overhead

4.2.2 Aarogyasri (Telangana & Andhra Pradesh)

Strengths:

- Highest coverage ceiling among state schemes (up to Rs. 10 lakh in Telangana)
- Pioneer of trust-based model, eliminating insurance company margins
- Extensive procedure coverage (1,672 procedures in Telangana)
- Well-established digital infrastructure for claims and verification
- Dedicated Aarogyamithra support system at hospitals
- Cochlear implant coverage for children (up to Rs. 6.5 lakh)

Weaknesses:

- No interstate portability restricts beneficiary mobility
- Exclusion of outpatient care leaves significant OOPEx gap
- Package rate disputes with private hospitals causing periodic disruptions
- Post-bifurcation divergence creates complexity
- Potential for fraud claims requiring constant vigilance
- Limited to BPL/low-income, excluding vulnerable non-BPL families

4.2.3 KASP & MEDISEP (Kerala)

Strengths:

- Only Indian scheme with chronic disease OPD coverage (KASP)
- Lowest out-of-pocket expenditure in India (35% vs. 62% national average)
- Dual-track approach covers both poor and government employees
- National portability for KASP beneficiaries
- Trust-mode implementation reducing intermediary costs
- Mobile clinics for tribal area outreach
- Moving toward true universal coverage with 2026 UHC scheme

Weaknesses:

- Significant fiscal burden on state exchequer due to OPD inclusion
- MEDISEP requires beneficiary premium contribution
- Coverage gaps for informal sector workers not covered by either scheme
- Limited to BPL/vulnerable for KASP (until 2026 UHC expansion)
- High per-capita health spending creates sustainability concerns

4.2.4 PM-JAY SEHAT (Jammu & Kashmir)

Strengths:

- Truly universal coverage regardless of income or socioeconomic status
- National portability allowing treatment across India
- No family size or age restrictions
- Integration with national PM-JAY infrastructure
- Telemedicine component for remote area access
- Coverage of all pre-existing conditions from day one

Weaknesses:

- Insurance-mode implementation adds intermediary costs
- Geographic and security challenges affect healthcare delivery
- Limited local tertiary care infrastructure necessitates interstate travel
- Enrollment and awareness gaps in remote/conflict-affected areas
- Potential confusion with other state schemes (Mukh Mantri SEHAT, etc.)
- Exclusion of outpatient care like most other schemes

4.3 Cross-Learning Opportunities

Based on the comparative analysis, several cross-learning opportunities emerge:

1. OPD Integration: Kerala's inclusion of chronic disease OPD follow-ups under KASP represents a best practice that could significantly reduce OOPE if adopted by other schemes. However, fiscal implications must be carefully modeled.

2. **Trust-Based Administration:** The Aarogyasri trust model, adopted by Kerala for KASP, eliminates insurance company margins and can direct more funds to actual care. States using insurance-company models (MJPJAY, SEHAT) could explore trust-mode transitions.
3. **Universal Coverage:** Jammu & Kashmir's universal approach under SEHAT, covering all residents regardless of income, represents the gold standard for UHC. Other states could phase toward universal eligibility, starting with vulnerable groups and expanding coverage.
4. **Enhanced Coverage Ceilings:** Telangana's Rs. 10 lakh coverage demonstrates that higher ceilings are feasible and necessary for catastrophic illnesses. States with lower ceilings (MJPJAY's Rs. 1.5 lakh) should consider periodic upward revisions.
5. **Digital Infrastructure:** Aarogyasri's end-to-end digital verification and claims system provides a template for reducing fraud and delays. All schemes should invest in robust digital platforms integrated with national health IDs.
6. **Support Systems:** The Aarogyamithra help desk model provides dedicated beneficiary navigation support that improves utilization and satisfaction. Similar patient navigation systems should be universal.
7. **National Portability:** SEHAT's and KASP's national portability addresses the critical need for healthcare access when beneficiaries travel or require specialized care unavailable locally. All schemes should work toward seamless interstate portability.

5. CHALLENGES AND LIMITATIONS

Despite significant achievements, government health insurance schemes in India face systemic challenges that limit their effectiveness and sustainability.

5.1 Beneficiary Identification and Exclusion Errors

A considerable proportion of genuinely poor households remains without insurance coverage. Studies indicate that 64% of the poorest households and 58% of poorer households lack any insurance coverage, primarily due to lack of awareness and identification issues. Conversely, leakage occurs when middle-class (71%) and affluent households (70%) access GSHI benefits meant for the poor. Linking SECC data with Aadhaar, conducting regular enrollment audits, and using community-based enrollment can reduce these errors.

5.2 Outpatient Care Exclusion

Most schemes, with the notable exception of KASP, exclude outpatient department (OPD) care, diagnostics, and medicines. National Health Accounts data shows that 60-70% of OOPE remains for non-hospitalized care. Chronic disease management requiring regular OPD visits, medications, and diagnostics forces households to bear significant costs despite having insurance.

5.3 Fragmentation and Lack of Integration

India's health insurance landscape remains fragmented, with different schemes operating in parallel with varying eligibility criteria, coverage packages, and implementation mechanisms. This creates confusion among beneficiaries and administrative inefficiencies. Integration of state and central schemes, while attempted in some cases (MJPJAY-PMJAY integration), remains incomplete.

5.4 Hospital Empanelment Quality

The quality of empanelled private hospitals varies significantly. Some hospitals engage in unnecessary procedures, package splitting, or upcoding to maximize reimbursements. Weak verification systems have identified 15% fake claims in some schemes. Simultaneously, delayed reimbursements to hospitals (up to 6 months in some cases) lead to service denials or demands for upfront payments from beneficiaries.

5.5 Primary Healthcare Neglect

The focus on secondary and tertiary care insurance has inadvertently diverted attention and resources from primary healthcare and preventive services. Health insurance schemes cannot substitute for robust primary care infrastructure, health workforce, and public health programs.

5.6 Fiscal Sustainability

States like Kerala that have expanded coverage to include OPD care face significant fiscal burdens. With India's health spending at only 2% of GDP (vs. WHO's 5% recommendation), sustaining comprehensive coverage while expanding benefits requires substantial additional resources. The tension between expanding benefits and maintaining fiscal sustainability is a constant challenge.

5.7 Geographic and Social Inequities

Rural-urban disparities in hospital network coverage persist. Tribal areas, remote regions, and conflict-affected areas (like parts of J&K) face particular challenges in accessing empaneled hospitals. Social factors including gender, religion, and caste influence scheme utilization, with women constituting 54% of OOPPE health spending despite being 49% of the population.

5.8 Awareness and Information Gaps

Low awareness in rural areas leads to under-utilization of schemes despite coverage. Many eligible families are unaware of their enrollment status, covered procedures, or how to access benefits. Complex documentation requirements and bureaucratic procedures further disadvantage illiterate and marginalized populations.

6. POLICY RECOMMENDATIONS

Based on the analysis of these four schemes and broader literature on publicly funded health insurance in India, the following recommendations are proposed:

6.1 Move Toward Universal Health Coverage

Jammu & Kashmir's SEHAT model of universal coverage regardless of income should be the long-term goal for all states. Transition pathways could include:

- Starting with vulnerable groups and progressively expanding to all residents
- Integrating multiple existing schemes into a single universal program
- Removing complex eligibility verification in favor of residency-based enrollment
- Learning from Kerala's 2026 "Health Protection for All" initiative

6.2 Integrate Outpatient Care

Following Kerala's KASP model, schemes should progressively include:

- OPD follow-ups for chronic diseases (diabetes, hypertension, cancer)
- Essential diagnostics and medications
- Preventive care and health screenings
- Telemedicine consultations for remote areas

This requires careful fiscal modeling and phased implementation to ensure sustainability.

6.3 Strengthen Primary Healthcare

Health insurance must complement, not substitute for, primary healthcare:

- Increase investment in PHCs and CHCs to at least 5% of GDP
- Ensure every village has functional primary care
- Integrate insurance schemes with primary care referrals
- Focus on preventive health camps and early detection

6.4 Harmonize State and Central Schemes

- Develop a unified national health benefits package with state flexibility
- Create seamless portability across all schemes nationwide
- Standardize empanelment criteria and quality standards
- Integrate beneficiary databases to prevent duplication and leakage
- Move toward a single national health insurance platform

6.5 Transition to Trust-Based Models

- Reduce dependence on commercial insurance intermediaries
- Adopt trust-based administration as pioneered by AarogyaSri
- Direct government-to-hospital payment mechanisms
- Eliminate insurance company margins to direct more funds to care

6.6 Enhance Digital Infrastructure

- Integrate all schemes with Ayushman Bharat Digital Mission (ABDM)
- Implement real-time claims processing and verification
- Use AI/ML for fraud detection and pattern analysis
- Create user-friendly mobile apps for beneficiary navigation
- Link with national health IDs for seamless verification

6.7 Improve Hospital Empanelment and Regulation

- Strengthen quality standards for empanelment
- Implement regular hospital audits and outcome monitoring
- Create patient feedback mechanisms
- Ensure timely reimbursement to hospitals (maximum 30 days)
- Penalize fraudulent practices while rewarding high-quality providers

6.8 Community Engagement and Awareness

- Conduct regular community health camps in underserved areas
- Train community health workers as scheme navigators
- Simplify documentation requirements
- Use local language communication and multimedia awareness campaigns
- Establish grievance redressal mechanisms at block level

6.9 Gender and Equity Focus

- Ensure women-headed households are prioritized in enrollment
- Provide special outreach for tribal, minority, and marginalized communities
- Address cultural barriers to healthcare access
- Monitor disaggregated data by gender, caste, and religion
- Include maternity and reproductive health services comprehensively

6.10 Sustainable Financing Mechanisms

- Increase central and state health allocations to 5% of GDP
- Explore sin taxes, health cess, and earmarked funding
- Develop performance-based financing linking outcomes to funding
- Create health insurance stabilization funds for high-cost years
- Encourage corporate social responsibility (CSR) participation in health infrastructure

7. CONCLUSION

Government-sponsored health insurance schemes have emerged as a critical pillar of India's healthcare system, providing financial protection to millions of vulnerable families who would otherwise face catastrophic health expenditure. The four schemes analyzed in this paper - MJPJAY in Maharashtra, AarogyaSri in Telangana and Andhra Pradesh, KASP/MEDISEP in Kerala, and PM-JAY SEHAT in Jammu and Kashmir - represent diverse approaches to achieving Universal Health Coverage within India's federal structure.

Each scheme demonstrates both significant achievements and persistent challenges. MJPJAY's long track record and extensive hospital network provide a model for geographic accessibility. AarogyaSri's trust-based administration and high coverage ceilings demonstrate the viability of state-led healthcare financing. Kerala's KASP, with its unique OPD coverage for chronic diseases, points toward the comprehensive coverage necessary to truly reduce out-of-pocket expenditure. Jammu and Kashmir's SEHAT scheme shows that universal coverage, regardless of income, is both feasible and necessary.

However, the analysis also reveals systemic gaps that require urgent attention. The exclusion of outpatient care in most schemes leaves a substantial portion of health expenditure uncovered. Fragmentation across multiple schemes with varying eligibility criteria creates confusion and inefficiency. Quality concerns in empanelled private hospitals, delayed reimbursements, and beneficiary identification errors undermine scheme effectiveness. Most critically, India's overall health spending remains inadequate at 2% of GDP, far below the levels needed for sustainable universal coverage.

The path forward requires a paradigm shift from fragmented, hospitalization-focused insurance toward integrated, comprehensive health coverage that includes primary care, outpatient services, and preventive health. Kerala's move toward "Health Protection for All" and J&K's universal SEHAT scheme provide roadmaps for this transition. Trust-based administration, robust digital infrastructure, and community engagement must replace insurance-company mediation and bureaucratic complexity.

As India aspires to be a developed nation by 2047, ensuring that no citizen faces financial ruin due to illness is not merely a policy option but a moral imperative. The schemes analyzed in this paper have laid the foundation; the task now is to build upon them to create a truly universal, equitable, and sustainable health protection system for all Indians.

The research concludes that while significant progress has been made, the journey toward Universal Health Coverage in India is far from complete. It requires sustained political commitment, increased public investment, administrative reform, and above all, a recognition that healthcare is a fundamental right, not a privilege.

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